

VETS MARKET INVESTIGATION

Summary of roundtable discussion on price lists held with veterinary professionals via MS Teams on 20 November 2025

Introduction

1. The following is a summary of points raised in a discussion between CMA case team members and representatives from organisations from across the sector,¹ including Large Veterinary Groups (**LVGs**), other main parties and independent veterinary practices.
2. Remedy 2b would require all veterinary businesses operating First Opinion Practices (**FOPs**), referral centres or crematoria to publish a standard price list, as outlined in the CMA's provisional decision report (**PDR**) part B.² In advance of the roundtable discussion, the CMA gave an overview of changes being considered following feedback on the PDR, including consideration of items on the list and of how items and information about them could be presented. In particular, a proposal for practices to use checkboxes rather than free text to indicate how services are delivered and priced was presented for discussion at the roundtable.
3. The remainder of the roundtable discussion focused on:
 - cross-cutting issues;
 - price list category: Consultation and preventative care;
 - price list category: Prescription, dispensing and administration;
 - price list category: Surgeries and treatments;
 - price list category: Diagnostics and laboratory tests;
 - price list category: End of life care; and

¹ The majority of which were currently or previously practising vets.

² [Provisional decision report: part B](#), 15 October 2025.

- price list category: Advanced treatments and procedures.

Cross-cutting issues

4. Several attendees were supportive of the CMA's suggestion to use checkboxes or drop-down menus, rather than free text, to present additional information about how services are delivered and priced in price lists. Some attendees cautioned that a high number of checkboxes could present a level of detail which could be overwhelming for the pet owner.
5. One attendee expressed concern that checkboxes or drop-down menus could be a barrier to having a contextualised care discussion with pet owners, which could result in people 'making a decision based on the lowest possible cost denominator'. Another attendee told us the addition of tick boxes to indicate technical details of the delivery of more complex services could drive the gamification of the price list and argued the price list was not the appropriate channel for this level of information. One attendee told us they were concerned at the length of the price list presented by the CMA. They stated that the use of checkboxes would not mitigate the risks they had identified with the price list: a race to the bottom, waterbed effects and hidden additional charges; and would not explain why certain components of fees were clinically necessary.
6. Some attendees said prices could be affected by who was performing the treatment and their level of qualification, while others said services or treatments delivered by a Registered Veterinary Nurse (**RVN**) should not necessarily be charged differently because the skillset is the same. There was general agreement that the price list should refer to RVN and Student Veterinary Nurse (**SVN**) staff delivery, where appropriate, and not to 'vet nurses'.
7. Attendees generally agreed that equipment could affect cost but that communicating the use of advanced equipment via the price list would be challenging and not necessarily understood by pet owners. One attendee stated that equipment did not necessarily affect outcomes in a way that was recognisable by the pet owner.
8. Attendees expressed a variety of views on the weight ranges proposed by the CMA. One attendee felt there were too few weight ranges, while another felt there were too many. One attendee suggested including specific example weights rather than a range, noting that in their clinic they charge based on whether the animal requires multiple staff to handle it.

Price list category: Consultation and preventative care

9. Attendees generally felt that any consultation time should be presented as an average or standard rather than a set time, as consultation lengths can vary depending on the situation.
10. One attendee said it could be helpful for customers to have more price information for out-of-hours treatment outside of the consultation price. Several attendees noted that the FOP often does not have access to their out-of-hours provider's prices. There was consensus that there should be a place to define the timeframe out-of-hours services should be used within, as this can vary across practices.
11. The CMA presented the suggestion to provide World Small Animal Veterinary Association (**WSAVA**) guidelines for defining 'core' vaccinations to support practice implementation.³ Several attendees said these guidelines could confuse implementation as they are not tailored to the UK. One attendee suggested referring to 'core' vaccinations, while another suggested the BSAVA Scientific Position Statement as an alternative.⁴
12. There was agreement that the animal health certificate price should make clear how many animals are included and the price for any additional animals.
13. One attendee commented that the CMA's current proposal for repeat consultations oversimplifies how prescription checks and repeat checks are managed. They said that making any follow up fall under the same level as a prescription check may not be appropriate as the time required may be different and explained that they have several pricing categories in their practice.

Price list category: Prescription, dispensing and administration

14. Attendees generally felt that the inclusion of dispensing and drug administration fees created complexity and did not add value for the customer, as these are not items the customer buys in isolation. One attendee said that if the CMA did choose to include these items, they should be more descriptive so that the customer can understand what the item refers to.

³ WSAVA (2024), 2024 guidelines for the vaccination of dogs and cats – compiled by the Vaccination Guidelines Group (VGG) of the World Small Animal Veterinary Association (WSAVA). Available at: [Vaccination Guidelines - WSAVA](#).

⁴ BSAVA (2021), Vaccination Position Statement. Available at: <https://www.bsava.com/position-statement/vaccination/>

15. One attendee was supportive of the inclusion of dispensing and drug administration fees as they felt it demonstrated to customers why medications are more expensive at a FOP than from an online pharmacy.
16. There was consensus that more clarity is needed on what prescription means. Attendees stated the following information would improve understanding: whether the price refers to a written prescription without the consult fee, how many medications are covered, the length of the prescription and whether it is repeatable.

Price list category: Surgeries and treatments

17. In terms of dental treatments, attendees indicated that it would be very difficult to give standard pricing for anything beyond routine dentistry. The CMA presented a proposal for pricing a single tooth extraction, but this was seen as impracticable as there was too much variation in cases. Attendees stated that oral assessment would be the most feasible inclusion and should include general anaesthetic and a scale and polish. Attendees said that dental x-rays were standard for some but not all practices in oral assessments, and generally agreed that this could be included as a checkbox.
18. Attendees felt that pain relief used during a procedure should be included in the main price rather than as a drop-down addition. One attendee said that separating the pain relief price from the headline treatment price could give a perception that this element of the treatment was optional and drive a focus on cost over animal welfare. One attendee stated that pain relief to go home with could be included as a drop-down extra, while others felt it was essential to include. One attendee suggested there should be defined criteria for what is included in each surgery and any practice that included extras at an additional charge would need to specify this on the price list.
19. Several attendees said that since anaesthesia and sedation are not bought in isolation, they should not be included on the price list. One attendee suggested that if they were to be included, they could be priced in blocks according to time or size.
20. Attendees agreed that the items in this category would require a description because practices may have different understandings of what is involved in a certain surgery. One attendee added that some customers might not understand what certain treatments involve and used the example of a laparoscopic spay.

21. Two attendees questioned whether it was appropriate to include laser therapy on the price list, noting there was mixed evidence on the effectiveness of this treatment.

Price list category: Diagnostics and laboratory tests

22. Attendees expressed differing views on the usefulness of providing a price for bundles of x-ray images. One attendee felt that for this to be implemented consistently, it would require further detail. Some attendees suggested that bundles could apply to a specific body part, for example a bundle of three x-ray images of the abdomen. Attendees agreed there should be an indication that sedation or anaesthesia would be required but there was disagreement on whether to include this in the headline price.
23. Attendees expressed a range of views on abdominal scans. Some felt that 'full' should be removed from 'full abdominal' as most FOPs did not offer this. One attendee suggested that a Point-of Care Ultrasound (**POCUS**) is comparable across practices, however others felt POCUS was not an appropriate inclusion. One attendee noted that the focus should be on common procedures because they are easier for vets to price and the information is more useful for consumers.
24. In response to the CMA's proposal to include two blood tests on the price list (pre-surgical and routine profiles), attendees noted that blood test profiles could vary significantly between practices. Attendees said technical definitions of the two items would need to be provided to support practice implementation of comparable tests, and some suggested additional inclusions for checkboxes. Attendees said that the tests should be clearly described so the customer could understand what was involved and included. One attendee added that 'pre-anaesthetic blood test' is a commonly used term and could be a useful comparator, but that all other blood tests are case dependent and therefore variable.
25. One attendee told us that the use of checkboxes for inclusions relating to the two blood tests would not be appropriate as there would need to be a significant number of checkboxes, risking confusion for the pet owner and making comparison difficult. Instead, they suggested the CMA set out a minimum core panel of blood tests for pre-surgical and routine blood tests, with a checkbox to indicate whether further tests are routinely included by a practice.
26. Several attendees expressed doubts about the practicability of providing prices for lab work because it was variable depending on the practice, use of external labs, and animal's needs. One attendee suggested an alternative

could be to list a professional fee, which would cover taking the sample and interpreting a test, leaving the specific test open to a conversation with the owner. However, another attendee noted that fees for interpreting tests could vary significantly so this may not be straightforward.

27. Regarding ear cytology and urine tests, there was consensus that FOPs typically deliver these internally so checkboxes to indicate use of external labs are not appropriate. One attendee added that microscopy should be removed from the basic urine screen.
28. Attendees said that FOPs often use referral centres for CT and MRI scans,⁵ or interpretation of the images and so these options need drop-down boxes to reflect the level of experience or specialism in interpretation.

Price list category: End of life care

29. One attendee suggested there should be weight banding for euthanasia and cremations. One attendee said that it should be made clear whether the price of an individual cremation includes the cost of an urn, casket or box.

Price list category: Advanced treatments and procedures

30. Attendees generally felt that items in this category would need specific definitions because practices would have different approaches to the surgeries and will provide different pain relief and lengths of hospitalisation. They said there should also be clarity on who was doing the surgery, for example a GP vet or an advanced practitioner.
31. Several attendees questioned the value of including this category in the price list, noting that as these items would be priced over £500, vets would need to provide a written estimate to customers.
32. One attendee expressed concern that the complexity of items in this category meant that owners would not be comparing like for like and they would not understand the nuances of the different offerings.
33. The CMA proposed introducing a surgery in addition to, or instead of, the existing list – pyometra.⁶ One attendee said that pyometra surgery could be usefully included if it were specified as ‘pyometra treated by ovarian hysterectomy’ and practices gave either the average or maximum price they

⁵ Computed Tomography (CT) and Magnetic Resonance Imaging (MRI) are advanced imaging techniques widely used in veterinary medicine.

⁶ Pyometra is a womb infection. It is a serious condition which is common in unneutered female dogs.

have previously charged. Most other attendees said that it would be difficult to price for because it was an unpredictable surgery with considerable variability.